

Fordyce's Spots (Fordyce Disease) and Montgomery's Tubercles

Fordyce's spots are commonly found in adults and appear as small, white or yellowish papules on the lips near the vermilion border or on the buccal mucosa. They may also occur on the genital mucosa, though this is rare. Montgomery's tubercles are similar lesions located on the areola of the breast, presenting as tiny papules arranged in a circular pattern near the periphery.

Both Fordyce's spots and Montgomery's tubercles represent ectopic sebaceous glands—Fordyce's in the oral mucosa and lips, Montgomery's in the areolar skin. Despite their atypical locations, these glands exhibit a histologically normal appearance and are best classified as hamartomas.

Histopathologically, both conditions show sebaceous glands or lobules situated in the upper submucosa (Fordyce) or dermis (Montgomery). Some lesions lack an associated sebaceous duct.

Treatment is generally unnecessary due to their benign nature, but cosmetic concerns may prompt simple surgical excision.

Sebaceous Hyperplasia

Sebaceous hyperplasia is a common benign tumor more frequently observed in males than females, typically appearing in middle age. Rare familial cases have been reported.

Clinically, it presents as solitary or multiple small (usually <3 mm), yellowish papules on the face—especially the forehead. A characteristic feature is a central umbilication or dell (see Fig. 119-19). These lesions are often mistaken for basal cell carcinomas.

Unusual variants include linear, zosteriform, giant, or diffuse forms, as well as lesions confined to the areola or vulvar region. Rhinophyma (discussed in Chapter 79) is now widely regarded as a specialized form of sebaceous hyperplasia.

Histologically, sebaceous hyperplasia is characterized by large, mature sebaceous lobules surrounding discrete, often dilated infundibula in the upper dermis. The infundibulum may contain keratinous debris, bacteria, and occasionally a vellus hair.

Treatment

Options include: - Electrodesiccation

- CO₂ laser ablation

- Cryosurgery

- Shave excision with curettage

Benign Neoplasms

Mantleoma

Mantleoma (also related to fibrofolliculoma) is a recently described benign cutaneous tumor showing differentiation toward the sebaceous mantle.

Clinical Features

- **Solitary mantleomas:** Sporadic, small facial papules that may mimic basal cell carcinoma or dermal nevi.
- **Multiple mantleomas:** Often associated with Birt-Hogg-Dubé syndrome. In such cases, fibrofolliculomas and trichodiscomas are now thought to represent different developmental stages of the same lesion—mantleoma.
- Lesions appear as numerous small papules on the head, neck, and trunk.

Histopathology

Histologic features vary: - Some lesions consist of cords and columns of undifferentiated epithelial cells radiating from an infundibulum.

- Others show interconnected epithelial cords containing sebocytes and sebaceous duct-like structures.

Management

- **Solitary lesions:** May be excised for diagnosis or cosmetic reasons.
- **Multiple lesions:** Managed similarly to fibrofolliculomas and trichodiscomas in Birt-Hogg-Dubé syndrome, with regular monitoring and symptomatic treatment.